STANDARD OPERATING PROCEDURE

SOP-OPS-001

Aeromedical Dispatch Operations

Receiving, triaging, assigning and dispatching aeromedical missions using the Medivac.ai Dispatch & Intake module.

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0.0 Document Control Summary

This Standard Operating Procedure (SOP) is a controlled document of the Royal Flying Doctor Service (RFDS) South Eastern Section. It is maintained within the Medivac.ai Document AI module. Printed or locally stored copies are uncontrolled and may not reflect the current approved revision. Confirm currency against the master record before use.

Version	Date	Author	Description of change
1.0	Jul 2026	Perplexity Computer	Initial draft issued for internal review.
1.1	Jul 2026	Perplexity Computer	Updated: Referral PDF attachment procedure, FTL Compliance Panel cross-references.

Approval and sign-off

Role	Name / Position	Signature	Date
Prepared by	Perplexity Computer		
Reviewed by	Operations Manager		
Approved by	Director of Operations		

> CONTROLLED DOCUMENT
> Amendments to this SOP require Director of Operations sign-off. Controlled copies are held exclusively in the Medivac.ai Document AI module. This document is reviewed on an annual cycle.

Contents

1.0 Purpose and Scope

1.1 Purpose

This Standard Operating Procedure defines the standard process for receiving, triaging, assigning, and dispatching aeromedical missions using the Medivac.ai Dispatch & Intake module. It establishes a consistent, auditable and safety-focused workflow that supports the timely and clinically appropriate movement of patients across the RFDS South Eastern Section network.

The procedure ensures that every retrieval and transport task — from a Priority 1 life-threatening activation through to routine Non-Emergency Patient Transport (NEPT) — is handled with the same rigour in tasking, risk assessment, crewing, and record-keeping.

1.2 Scope

This SOP applies to all RFDS SE Dispatchers and Duty Pilots, and to any personnel performing dispatch, tasking, or mission-coordination duties on behalf of the Section, including on-call and after-hours staff. It governs all fixed-wing aeromedical activity coordinated through Medivac.ai, including:

- Emergency retrievals and inter-hospital transfers (Priority 1, 2 and 3);
- Non-Emergency Patient Transport (NEPT) tasks scheduled and batched through the platform;
- Crew assignment, fatigue management, and operational risk assessment activities associated with each mission; and
- Mission monitoring, completion, and record closure prior to shift handover.

1.3 Exclusions

This SOP does not replace the RFDS SE Operations Manual, aircraft flight manuals, or clinical care protocols. Where a conflict exists between this SOP and an approved operations manual, regulatory instrument, or a lawful direction of the Pilot in Command, the higher-order document or the safety of flight prevails.

> PRINCIPLE
> No mission is dispatched without a completed tasking record, confirmed legal crew, and — for Priority 1 and Priority 2 missions — a completed Operational Risk Assessment in Medivac.ai.

2.0 References

This SOP shall be read in conjunction with the following governing documents and regulatory instruments. Users must ensure they refer to the current approved version of each reference.

Ref.	Document	Type / Authority
R1	RFDS SE Operations Manual	Internal — Operations
R2	CASA Part 135 — Air transport operations (smaller aeroplanes)	Regulatory — CASA
R3	RFDS Pilots Enterprise Bargaining Agreement (EBA) 2025	Industrial Instrument
R4	RFDS Nurses Enterprise Bargaining Agreement (EBA) 2023	Industrial Instrument
R5	Medivac.ai User Manual	System — Vendor
R6	RFDS SE Communications Manual	Internal — Operations

DUTY AND FATIGUE LIMITS

Crew duty limitations applied within this SOP are derived from CASA Part 135 flight and duty time provisions (R2) and the applicable RFDS EBAs (R3, R4). Where the instruments differ, the more conservative limit applies. The Medivac.ai Duty & FRMS module encodes these limits and flags conflicts automatically.

3.0 Definitions and Abbreviations

The following terms and abbreviations apply throughout this SOP.

Term	Definition
NEPT	Non-Emergency Patient Transport — non-urgent transport that is batch scheduled rather than individually activated.
IFR	Instrument Flight Rules — rules governing flight by reference to instruments in controlled or instrument meteorological conditions.
ORA	Operational Risk Assessment — the structured pre-mission risk evaluation completed in Medivac.ai.
FRMS	Fatigue Risk Management System — the system and controls used to manage crew fatigue and duty limits.
Referring Facility	The hospital or medical centre requesting patient transport.
Receiving Facility	The destination hospital or health service to which the patient is transported.
PIC	Pilot in Command — the pilot responsible for the operation and safety of the aircraft during flight.
Wheels-up	The time at which the aircraft becomes airborne, used as the key dispatch-performance reference.

3.1 Priority Classification

All tasking is classified against the RFDS SE Priority Classification. The priority level drives the target wheels-up performance standard and the level of risk oversight required.

Priority	Category	Definition	Target wheels-up
P1	Immediate	Immediate life-threatening condition.	Within 30 min
P2	Urgent	Urgent condition requiring prompt transport.	Within 2 hours
P3	Non-urgent	Non-urgent condition; clinically stable.	Within 24 hours
NEPT	Non-emerg.	Non-emergency transport; batch scheduled.	Batch scheduled

4.0 Responsibilities

The following roles and accountabilities apply to dispatch operations. All personnel must operate strictly within the limits of their role, currency, and authority.

4.1 Dispatcher

Receives requests through all approved channels; triages and classifies each task against the RFDS SE Priority Classification; assigns available and legal aircraft and crew; issues mission briefings; monitors missions to completion; and maintains complete and accurate mission records in Medivac.ai.

4.2 Duty Pilot

Confirms aircraft availability and serviceability; completes aircraft performance planning; and accepts or declines missions based on operational, weather, and regulatory considerations. The Duty Pilot exercises Pilot-in-Command authority for flight-safety decisions.

4.3 Senior Base Pilot

Approves high-risk or operationally complex missions; acts as the primary escalation point for the Duty Pilot and Dispatcher; and provides operational guidance where a mission exceeds normal dispatch parameters or returns a Medium ORA outcome. The SBP's mission-approval role is now supported by the FTL Compliance Panel in the SBP Portal (Overview tab), which surfaces at-risk pilots by base (3HR WARN / 2HR WARN / HARD BLOCK) and provides a quick link to the FTL Compliance Engine. The SBP should review this panel at the start of each shift and before approving any crew assignment.

4.4 Flight Nurse / Clinical Crew

Confirms clinical requirements and patient-care needs; specifies medical equipment, consumables and escort requirements; and advises the Dispatcher of any clinical factors affecting crewing, aircraft configuration, or timing.

4.5 Operations Manager

Provides overall operational oversight; acts as the after-hours escalation authority; and approves High-risk missions arising from the Operational Risk Assessment. The Operations Manager is accountable for the effective functioning of dispatch operations.

4.6 Responsibility Summary (RACI)

Activity	Dispatcher	Duty Pilot	Snr Base Pilot	Ops Mgr
Receive & log request	R/A	I	I	I
Triage & priority	R/A	C	I	I
Aircraft / crew assignment	R	A	C	I
Operational Risk Assessment	R	C	A (Med)	A (High)
Mission acceptance	C	R/A	C	I

Activity	Dispatcher	Duty Pilot	Snr Base Pilot	Ops Mgr
Escalation	R	R	A	A

R = Responsible | A = Accountable | C = Consulted | I = Informed

5.0 Dispatch Process — Step by Step

The dispatch workflow proceeds through eight sequential stages. Each stage is completed and recorded in Medivac.ai before the mission advances. The stages are summarised below and detailed in the following sub-sections.

Stage	Activity	Medivac.ai module
5.1	Receiving a request	Dispatch & Intake
5.2	Triage & priority classification	Dispatch & Intake
5.3	Aircraft & crew assignment	Aircraft Status; Crew Roster; Duty & FRMS
5.4	Operational Risk Assessment	ORA
5.5	Mission briefing	Dispatch & Intake
5.6	Launch & monitoring	NSW Flight Map
5.7	Mission completion	Tech & Journey Log
5.8	NEPT batching	NEPT Tasking

5.1 Receiving a Request

All requests are received via phone, secure messaging, or direct entry into the Medivac.ai Dispatch & Intake module. The Dispatcher completes the following steps:

1. Answer or acknowledge the request through the approved channel and confirm the identity and authority of the requesting clinician or facility.

1a. Attach Referral PDF (if provided): If the requesting facility provides a referral document, click the ‘Attach Referral PDF’ button in the Dispatch & Intake form. Confirm the filename displayed is correct — use the remove (×) control if the wrong document was attached. The attachment clears automatically on form reset. Ensure the attached PDF is visible to all clinical crew before departure.

2. Log the requesting (referring) facility in Medivac.ai.
3. Record patient details: age, weight, diagnosis, and requested priority.
4. Record the destination (receiving facility) and confirm whether a clinical escort is required.
5. Confirm whether the patient has any special handling requirements (e.g. bariatric, neonatal, infectious, or ventilated).
6. Allow Medivac.ai to automatically generate and assign the mission Reference Number.
7. Read back the captured details to the requesting clinician to confirm accuracy.

> REFERENCE NUMBER
> A unique mission Reference Number is generated automatically by Medivac.ai at the point of intake. This number is quoted in all subsequent communications, briefings, and records for the mission.

5.2 Triage and Priority Classification

The Dispatcher classifies each request against the RFDS SE Priority Classification (P1–P3 / NEPT, see Section 3.1) and confirms the clinical basis for that classification:

1. Apply the RFDS SE Priority Classification based on the clinical information provided.
2. Where the priority is unclear or the clinical picture is ambiguous, confirm the priority directly with the referring clinician before proceeding.
3. Document the clinical summary and the agreed priority in the mission record.
4. Escalate to the Senior Base Pilot early where the task is likely to be high-risk or complex.

5.3 Aircraft and Crew Assignment

The Dispatcher assigns an available, serviceable aircraft and a legal, available crew, confirming compliance with all duty and fatigue limits before assignment:

1. Check the Aircraft Status module to identify available and serviceable aircraft appropriate to the mission profile.
2. Check the Crew Roster and the Duty & FRMS module to identify available and legal crew.
3. Confirm that crew duty hours do not exceed the applicable CASA Part 135 or EBA limits, applying the more conservative limit where these differ.
4. Assign the aircraft and crew in Medivac.ai; the system automatically flags any fatigue or duty conflicts arising from the assignment.
5. Resolve any flagged conflict before confirming the assignment — do not override a fatigue flag without documented Senior Base Pilot or Operations Manager approval.

> AUTOMATED FATIGUE FLAGS
> Medivac.ai flags fatigue and duty conflicts automatically at the point of assignment. A flagged conflict is a hard stop: the mission must not proceed on that crewing until the conflict is resolved or an authorised override is recorded.

> FTL COMPLIANCE CHECK
> Before confirming crew assignment, verify the assigned pilot's FTL status in the Senior Base Pilot Portal — FTL Compliance Panel. A pilot at HARD BLOCK status (≤ 60 min FDP remaining) cannot be legally assigned. Pilots showing 2HR WARN or 3HR WARN status should be flagged to the SBP for review before assignment is confirmed.

5.4 Operational Risk Assessment (ORA)

An Operational Risk Assessment is completed in Medivac.ai for all Priority 1 and Priority 2 missions. The ORA evaluates the following factors:

- Weather at the destination and en-route, including forecast trends;
- Pilot currency and recency for the mission profile;
- Medical complexity of the patient and associated crewing requirements;
- Time of day, including night operations and circadian factors; and
- Alternate aerodrome availability and suitability.

The ORA produces a risk score that determines the required approval level:

ORA outcome	Approval authority	Action
Low	Dispatcher	Dispatcher may approve and proceed.
Medium	Senior Base Pilot	Senior Base Pilot review and approval required before dispatch.
High	Operations Manager	Operations Manager approval required before dispatch.

5.5 Mission Briefing

The Dispatcher issues a verbal brief to the assigned crew via phone or radio prior to launch. The brief follows a consistent structure and is confirmed within Medivac.ai:

1. Deliver the verbal brief to the assigned crew, quoting the mission Reference Number.
2. Brief the patient details, destination, and weather summary.
3. Brief the ORA outcome and any conditions attached to the approval.
4. Brief any special considerations (clinical, operational, or aerodrome-specific).
5. Obtain and record the crew's confirmation of acceptance in Medivac.ai.

> BRIEF CONTENT
> Every mission brief includes, as a minimum: patient details, destination, weather summary, ORA outcome, and special considerations. Crew acceptance is recorded in Medivac.ai before launch.

5.6 Launch and Monitoring

Once the crew accepts the mission, the Dispatcher records the launch and actively monitors the mission through each phase of flight:

1. Record the wheels-up time in Medivac.ai as the mission becomes airborne.
2. Monitor the flight via the NSW Flight Map throughout the mission.
3. Update the mission status at each phase as it is reached.

The mission status is progressed through the following sequence:

Dispatched → Airborne → On Scene → Patient Loaded → En Route → Landed → Complete

5.7 Mission Completion

On completion of the mission, the crew and Dispatcher finalise the record and flag any safety matters:

1. The crew completes the Tech & Journey Log entry post-flight.
2. The Dispatcher closes the mission record in Medivac.ai, confirming all mandatory fields are complete.
3. The Dispatcher flags any incidents or near-misses for Safety reporting.

> DATA COMPLETENESS
> A mission record cannot be closed until all mandatory fields are complete. Incomplete records are escalated at shift handover (see Section 9).

5.8 NEPT Batching

Non-Emergency Patient Transport tasks are accumulated and scheduled as daily batches to optimise aircraft utilisation and routing:

1. Accumulate NEPT requests through the day and batch them daily at 1400 local time.
2. Use the NEPT Tasking module to group tasks by region, optimise routing, and assign crews.
3. Confirm all NEPT missions with the receiving facilities 24 hours prior to the scheduled movement.

> NEPT BATCH CUT-OFF
> The daily NEPT batch is compiled at 1400 local. Requests received after the cut-off are, by default, scheduled into the following day's batch unless re-prioritised on clinical grounds.

6.0 After-Hours Dispatch

Outside standard operating hours, dispatch functions are maintained through the On-Call Dispatcher, who retains full operational capability via mobile access to Medivac.ai.

- After-hours requests are directed to the On-Call Dispatcher through the approved channels.
- The On-Call Dispatcher has full Medivac.ai access via mobile and performs the complete dispatch workflow described in Section 5.
- All after-hours Priority 1 activations require Senior Base Pilot notification within 15 minutes of activation.

> AFTER-HOURS P1 NOTIFICATION
> For any after-hours Priority 1 activation, the On-Call Dispatcher must notify the Senior Base Pilot within 15 minutes. The notification is recorded in the mission record in Medivac.ai.

Scenario	Primary contact	Escalation / timing
After-hours request (any priority)	On-Call Dispatcher	Standard workflow (Section 5)
After-hours P1 activation	On-Call Dispatcher	Notify Senior Base Pilot within 15 minutes
After-hours escalation	Operations Manager	Per Section 8 — Escalation

7.0 Communication Standards

Consistent, disciplined communication underpins safe dispatch operations. The following standards apply to all mission-related communication.

- All mission-critical communications are logged in Medivac.ai against the relevant mission Reference Number.
- ICAO standard phraseology is used for all Air Traffic Control (ATC) coordination.
- Radio procedures follow the RFDS SE Communications Manual (R6).

7.1 Key ICAO Aerodrome Identifiers

The following ICAO location indicators are used across the network. Dispatchers should be familiar with these identifiers for tasking and coordination.

Aerodrome	ICAO identifier
Dubbo	YSDU
Broken Hill	YBHI
Bankstown	YSBK
Essendon	YMEN
Launceston	YMLT

8.0 Escalation

Escalation ensures that safety-significant events and operational disruptions receive prompt attention from the appropriate authority.

8.1 Safety Events

Any safety event, near-miss, or mission abort requires immediate notification to the Operations Manager and the Safety Officer. The event is recorded in Medivac.ai and progressed through the Safety reporting process.

8.2 Aircraft Unserviceability Mid-Mission

Where an aircraft becomes unserviceable during a mission, the Dispatcher coordinates with Engineering via the Medivac.ai Engineering module and works with the Duty Pilot and Senior Base Pilot to re-plan or re-crew the task as required.

Trigger	Immediate action	Notify
Safety event / near-miss	Record in Medivac.ai; initiate Safety report.	Operations Manager; Safety Officer
Mission abort	Record in Medivac.ai; secure patient care continuity.	Operations Manager; Safety Officer
Aircraft unserviceable mid-mission	Coordinate via Engineering module; re-plan / re-crew.	Engineering; Duty Pilot; Snr Base Pilot

IMMEDIATE NOTIFICATION
Safety events, near-misses, and mission aborts are non-discretionary escalations. Notify the Operations Manager and Safety Officer immediately — do not defer to shift handover.

9.0 Quality and Compliance

Quality assurance ensures dispatch records are complete, missions are auditable, and performance is measured against defined standards.

- All missions must have complete Medivac.ai records before shift handover.
- A random audit of dispatch records is conducted quarterly.
- Performance is measured against the key performance indicators below.

9.1 Key Performance Indicators

KPI	Description
P1 wheels-up compliance	Proportion of Priority 1 missions achieving wheels-up within the 30-minute target.
Mission completion rate	Proportion of accepted missions completed as tasked.
Data completeness score	Proportion of mission records with all mandatory fields complete at closure.

HANDOVER GATE
Complete Medivac.ai records are a prerequisite for shift handover. Any incomplete record must be resolved or formally handed over with a documented reason and owner.

10.0 Document Control

This SOP is a controlled document maintained within the Medivac.ai Document AI module.

- This SOP is reviewed annually.
- Amendments require Director of Operations sign-off.
- Controlled copies are held in the Medivac.ai Document AI module.

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